



GUIDELINE

Leave Management in Community Adult Mental Health Services

Scope (Staff): All NMHS Community Adult Mental Health Staff

Scope (Area): All NMHS Community Adult Mental Health Staff

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1. Aim

This guideline outlines leave management principles, responsibilities and strategies within Community Adult Mental Health Services for leave to be processed in a fair, timely, and efficient manner to meet the needs of both the individual and the service.

2. Background

The scheduling and processing of leave applications for all staff members is an organisational requirement.

The Business Rules ensures:

- staff members receive their leave entitlements allowable to them under the various [industrial awards and agreements](#) in an efficient and timely manner; and
- leave is scheduled in a fair and equitable manner ensuring efficient and effective functioning of the service in meeting the needs of our consumers.

To assist this process, Community Adult Mental Health have established rules to:

- Identify adherence requirements of staff when applying for the various categories of leave.
- Identify the approval process in accordance with the [NMHS Leave Management Policy](#).
- Ensure maximum opportunities are provided to acquit leave, on a yearly basis, before it becomes excess leave.

3. Risk

Non-compliance with this guideline may result in of inequitable allocation of leave and excess leave accruals. In addition to impact on

Culture	☒	Organisational Development	☐
Emergency Management	☒	Policy & Legislative Compliance	☐
Fiscal Responsibility	☒	Quality of Patient Care	☒
Governance & Accountability	☒	Research & Innovation	☐
Information Communications Technology	☐	Staff Safety & Wellness	☒
Facility Management	☐	Workforce Planning	☒
Medical Equipment	☐	Other	☐

4. Key Points

- It is the responsibility of all staff members to ensure all leave is submitted on a HSS L1/L2 form to their line manager.
- L1/L2 forms will list one period of continuous leave only.
- It is the responsibility for each staff member to check the team leave planner before submitting leave applications to ensure their application falls within the maximum leave allocations (see section 5 below).
- Senior staff who approve leave applications are responsible for ensuring that minimum numbers are adhered to, as below. Consideration should be given to the psychological wellbeing and fatigue management of remaining staff.
- Leave applications should be approved or declined within two (2) weeks of being sent to the Service Coordinator.
- Every effort will be made to backfill periods of leave of more than 3 weeks. Suitable notice needs to be given to the Service Coordinator for backfill to be arranged.
- Consideration will be given to the provision of higher duties in maintaining senior team members.
- For all periods of planned absence, staff should ensure their caseload has been handed over as per the Department of Health's [Clinical Handover Policy](#) and [Clinical Handover Matrix](#).
- Staff members will check the Roster and Leave Planner to ensure that their application fits within the following guidelines applicable to all staff leave unless otherwise advised by the Service Coordinator, Service Manager or Coordinator of Nursing.

5. Maximum Leave Allocation

Leave applications and approvals within community teams should not leave the team with less than 60% of the normal complement of staff, or 50% within a small team. This number includes staff:

- on workers compensation leave;
- rostered day's off (RDO's);
- uncovered long term Personal leave; and
- unfilled / backfilled positions.

6. Procedure for Applying for Leave and Accrued Day Off (ADO)

6.1 Annual Leave & ADO's:

The following process for leave applications and ADO's will apply:

- Staff are to check leave entitlements to ensure adequate leave has been accrued.
- Staff are to discuss leave requests with supervisor and clinical/clerical team during the Multidisciplinary Team (MDT) meeting using the leave planner as a guide. Discussions and decisions are to be recorded in team minutes.
- Staff are to be mindful of equity of leave approvals, particularly around peak periods of leave, skill mix and seniority of remaining staff (i.e. avoiding having multiple seniors off at one time or all nursing staff at one time)
- Once endorsed by your MDT – Staff member is to submit an application to line manager utilising the L1 (Medical) or L2 (Non-Medical) leave form. **Paper forms will not be accepted.** The line manager should not support leave request unless there is evidence that the application has been discussed and supported with the clinical/clerical team and is within the staffing limitations stated above.
- The Line Manager (as below) should note an authorisation in the comments box of the form and then send form to the Service Coordinator or Head of Clinical Services (HOCS) for final endorsement. The Service Coordinator will then forward to RoStar admin (G3) or HSS and save the form in W drive.
- Annual leave applications are preferred six (6) weeks in advance and should not be requested during a published roster, unless in exceptional circumstances e.g. bereavement or serious illness of a close relative (for single leave days see below); this enables time for rosters to be published in line with EBA requirements.
- Requests for ADO's or other single leave days should give at least one (1) week's notice unless otherwise agreed with Service Coordinator. If the roster has been published, and the leave is for an out-of-hours shift, it is the responsibility of the requestor to find a suitable replacement.
- ADO's/Time Off in Lieu (TOIL)/Personal Leave of **less** than one week should be entered directly into the roster by the Service Coordinator/Admin Assistant and the L2 form saved on the W drive/ TRIM.
- ADO's/TOIL/Personal Leave of **more** than one week on previous or current roster to be entered directly into RoStar by the Service Coordinator/Admin Assistant and the L2 form saved on the W drive
- Annual Leave or Long Service Leave in previous roster will be entered directly into RoStar by the Service Coordinator/Admin Assistant and the L2 form saved on the W drive.

- Any leave discussions or decisions made within the business section of the MDT should be recorded in the minutes of the MDT.
- Line Managers are responsible for ensuring that L1/L2 forms are received and then forwarded to Service Coordinator as follows:
 - Admin to Senior Admin Officer
 - Base grade HSU to Senior HSU
 - Level 2 nursing to SRN 3
 - Registrar/Medical officer to Consultant Psychiatrist
 - Senior Admin Officer to Service Coordinator
 - Senior HSU staff to Service Manager
 - SRN 2/3 Nursing to Coordinator of Nursing
 - Service Coordinator to Service Manager
 - Consultant Psychiatrist to Head of Clinical Service
 - Service Manager to Co-Director of Adult Program

The Line Manager will authorise and forward form to the Service Coordinator for final approval following consideration of staffing levels and staff mix

- The relevant senior staff will forward all leave forms to the nominated Service Coordinator.
- All requests made at short notice (less than 72 hours) will be referred to the Service Coordinator for approval.
- Other than emergency periods of leave, the granting of any form of leave will be on the provision that overtime is not incurred to the service.

6.2 Peak Periods e.g. School Holidays, Easter, Christmas and New Year:

- Peak periods are not classified as low activity periods.
- The decision to grant leave is **not** based on a “first come, first served” basis.
- Staff are to be mindful of equity of leave approvals, particularly around recency and frequency of leave over peak periods, skill mix and seniority of remaining staff (i.e. avoiding having multiple seniors off at one time or all nursing staff at one time)
- The team will discuss and reach a provisional agreement in accordance with the regular leave request process. However, should the team be unable to reach agreement, the Service Coordinator will facilitate a final decision.
- The decision will be made considering the needs of the service as well as the needs of the employee and previous history of taking leave during peak periods.
- This is to ensure that every staff member has an equitable opportunity to have leave during a peak period.
- **Staff members should be mindful that travel arrangements and accommodation bookings should not be committed to until your leave request is approved.**

6.3 Long Service Leave:

The same procedure for applying and approving Long Service Leave that has been adopted for Annual Leave applications will apply.

Staff should check their award and ensure that they are submitting applications within the required timeframe.

6.4 Sick leave, Personal Leave and Carers Leave:

All [industrial awards](#) specify the responsibilities of employees when taking sick leave. Please see your award and ensure that you abide by the conditions set out, regarding certification of sick leave.

The following procedure for Sick Leave, Personal Leave, or Carers Leave applications will apply:

- Reporting: As a minimum, staff will telephone the reception at their respective clinic at 8:30am and advise they will not be attending for their shift. The staff member should indicate a possible date of return to work.
- Each morning, the reception staff will collate a list of staff members taking sick leave and report absences to all staff via email.
- Where practicable, staff should speak directly to their line manager to report their leave.
- Upon returning to work, the employee will complete the leave form (L1/L2) and submit this to their line manager on the same day of recommencing duty. Staff should submit medical certificates with their application where applicable.
- The line manager should reconcile the employee's application form to ensure that the correct leave category and number of days are recorded and the leave application and supporting documents should be emailed to the Service Coordinator.

6.5 Professional Development Leave:

- To monitor staffing numbers in each team, all courses will be recorded in RoStar either as professional development leave (as per HSU/ANF/AMA awards) or within the footnotes.
- **Approval is required for all courses.**
- An L1/L2 along with information about the course being applied for is to be provided for all courses except:
 - Core competency – mandatory training (CPR, M Handling, F&S, PART).
 - Exchange visits to NMCADS.
 - Any training as directed by your line manager.
- Some L1/2 forms may not be forwarded to HSS however they provide evidence of approval.
- When completing an L1/L2 for Professional Development leave, it should contain the following information in the comments box:
 - confirmation of Line Manager approval
 - course attendance dates and times
 - confirmation of who will provide funding
- All disciplines forms will be kept on file within a secure folder in the W drive.

6.6 Purchased Leave (PL):

Staff may apply to purchase leave as per [industrial award](#) to the Service Coordinator who will submit, along with supporting documentation, to the Service Manager/Coordinator of Nursing for approval.

Requests will be determined on a case-by-case basis endeavouring to ensure equity of allocation of purchased leave based on operational needs, personal leave accrual/balance and demand for PL.

It should be noted that, while every staff member has leave entitlements under their specific award, the taking of that leave is an agreement between the employer and the employee, except

where excess leave has accumulated, and the employer can direct the employee to take leave.

There are rare occasions when operational needs will require cancellation of booked leave. This will be in exceptional circumstances and will be decided in conjunction with the Service Manager and or Coordinator of Nursing.

7. Paying out leave (L13 Application to Cash out Leave entitlements)

A request to pay out leave (HSS form L13) may be submitted to the Service Coordinator for submission to the Co-Director. Applications should include a rationale for the request.

8. Right of appeal:

If any staff member is concerned about a decision made in relation to this leave policy, they are encouraged to discuss with their line manager or Service Coordinator in the first instance in accordance with the [MP 0116/19 – DoH Grievance Resolution Policy](#).

9. Compliance, monitoring and evaluation

The Service Coordinator will monitor excess leave and report any issues to the Operational Community Adult Co Director

Related internal policies, procedures, guidelines and forms
NMHS Leave Management Policy WA Department of Health Clinical Handover Policy WA Department of Health Clinical Handover Matrix WA Department of Health Grievance Resolution Policy L1 (Medical) L2 (Non-Medical)

Policy Sponsor	Operational Co-Director Community MH Adult Services				
Policy Contact	Operational Co-Director Community MH Adult Services				
Date First Issued:	11/08/2023	Last Reviewed:	01/09/2025	Review Date:	01/09/2028
Approved by:	Community Adult Leadership Committee			Date:	28/07/2025
Endorsed by:	Operation Co-Director Community MH Adult Services			Date:	01/09/2025
NSQHS Standards Applicable:	☒  Std 1: Clinical Governance ☐  Std 2: Partnering with Consumers ☐  Std 3: Preventing and Controlling Healthcare Associated Infection ☐  Std 4: Medication Safety ☐  Std 5: Comprehensive Care ☐  Std 6: Communicating for Safety ☐  Std 7: Blood Management ☐  Std 8: Recognising and Responding to Acute Deterioration				
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Document Version Control			
Date	Version	Author	Notes
11/08/2023	1.0	Co-Director Community MH Adult Services	Initial endorsement
01/09/2025	2.0	Service Manager Community MH Adult Services	Review and update of current practice

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